SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cemiplimab for advanced cutaneous squamous cell cancer

NCCP 00812b: metastatic or locally advanced cutaneous squamous cell carcinoma when curative surgery or curative radiotherapy is not appropriate. This guide covers skin SCC, not the separate cervical-cancer indication. · NCCP 00812 · version 3a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Cemiplimab helps immune cells act against advanced cutaneous squamous cell cancer when curative surgery or radiotherapy is not suitable. The team reviews cancer control, symptoms, side effects and your priorities to decide whether to continue.

Schedule
Cemiplimab is given by IV infusion on Day 1 every 21 days. The usual source dose is 350 mg over 30 minutes; the prescription and visit checks confirm your treatment. There is no fixed cycle total in this regimen: treatment continues while the team considers it beneficial and tolerable.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cemiplimab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Skin reactions and rare severe skin injury
- ☐ Rare heart, muscle, neurological and eye inflammation
- ☐ Infusion reactions
- ☐ Some immune effects need long-term treatment

Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00812 v3a.

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